

Weekend Healthcare Pulse: Prescription for AI, Google Health takes the lead

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The numbers speak for themselves: 50 million health questions a day for Microsoft (covered by M. Moerdler) and more than one billion for Google (covered by M. Shmulik - [Check Up 2026, Google Analytics](#)). Beyond the breadth and depth of the questions related to health, reflecting limitations in health knowledge, clinical practices and behavior among patients, it highlights a sound commitment from these two companies to using AI for health matters. That said, the two most common search engines, Google and Microsoft, are tackling personal health from different angles: Copilot Health is a dedicated, secure space inside Copilot that can integrate and interpret an individual's health data, while Google Health is a collection of platforms that can be accessed by various stakeholders via the cloud. The aim of this weekender is to provide an overview of usage, give readers a greater understanding of both platforms and highlight some clear limitations.

HEALTH: A STAGGERING AMOUNT OF DATA

Microsoft claims to respond to 50 million consumer health questions a day. **Google has reiterated that it receives one billion+ health questions a day** out of c. 14 billion total queries. These are striking figures. Hema Budaraju, head of AI Analytics at Google Search, talked of the following insights at the Google Check Up 2026 event:

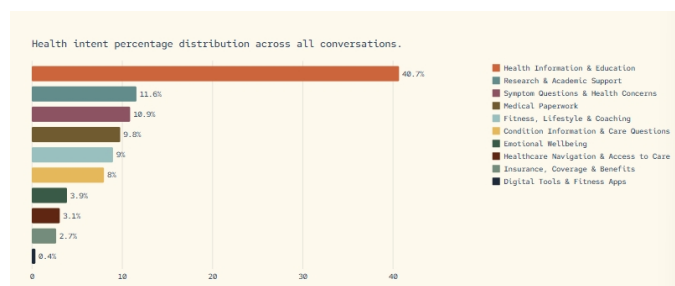
- Evolution towards a defined and specific health matter vs general questions around a disease, health requests are three times the average length of a Google request and give more context.
- Personalization of the request: it is no longer about "headaches" for instance, but about describing symptoms and behavior in order

to get more insight/a more elaborate response from the tool to a medical question that directly relates to the individual.

Against this backdrop, we note that there are a **cumulative one trillion health videos available globally on YouTube**, that half of the world's health workers are already using YouTube for clinical content according to Dr Graham, Global Head of YouTube Health, and that YouTube is also now used a learning tool in medical studies.

WHAT HEALTH DATA ARE PEOPLE LOOKING FOR?

EXHIBIT 1: Copilot health requests of January 2026 still focus more on education than on treatment paths



Source: Copilot health

Microsoft's Copilot publicly shares underlying requests per type of search which turn around information for more than 40% followed by research and academic support and direct symptoms questions. The personal lifestyle and sports coaching themes are clearly showing less engagement for Copilot. What is interesting is that most requests still focus on only the first layer of AI, i.e.

getting reliable and extensive data, as opposed to the next step of discussing ways to alleviate symptoms.

WHAT THESE DATA MEAN FOR AI ENGINES AND PLAYERS?

We are witnessing a shift away from the compilation or linear approach (tools/data), to a multi-modal approach that can solve queries using multiple interfaces and inter-operabilities (where devices and AIs interact).

In parallel, there is a need for more enriched data and more powerful apps in the most common therapeutic areas, such as sleep apnea and metabolic syndrome/ diabetes, within FitBit's Personal Health Coach. New algorithms to be launched this year should increase the accuracy of the sleep apnea data (nap detection, sleep interruptions, restlessness at night) by 15% and improve the prediction of insulin resistance by connecting continuous glucose monitoring (CGM) to Google's Health Connect for instance. The multi-modal approach also incorporates access to medical records through Athenahealth and CMS as well as Health100 from CVS.

And the more data there is, the higher the marketing fees the model can generate. So far, Google does not allow ads to appear alongside its AI Health summary responses. While the regulation remains complex and country-specific, we can imagine a monetization system based not on the number of queries but more on the degree of sophistication, whereby the response sends more traffic to external commercial partners. It is not simply about quantity but increasingly about the level of precision and depth of the AI response and model.

GOOGLE HEALTH: INCREASINGLY HARD TO COMPETE WITH IT

Google Health is a collection of platforms that can be accessed by various stakeholders via the cloud. It uses the Google Medical Imaging suite, Gemini, to provide enlarged "education", and aggregates data collected via Fitbit (now named Google Health), Pixel, various other health apps, and wearables, into a single app, Health Connect by Android. The idea for Google is to "find new ways to meet people where they are on their health and wellness journey, whether that is through Search, YouTube, wearables or other devices".

The cornerstone of Google Health is Gemini, which is able to navigate and reason in a multi-modal health landscape using Large Sensor models to navigate sensor data and the personal health large language model (PH-LLM), and MedGemma, an open model for medical text and image comprehension dedicated to physicians. Google Deepmind has also created AlphaFold to predict the 3D shape of the protein. The partnership with Isomorphic Labs ([Weekend Healthcare Pulse: An update on the latest in EU VC funding see page 5](#)) has paved the way to drug discovery, as well as further collaboration with large pharma/ biotech companies

such as the one with Roche.

All the data provides a baseline against which Google is now deploying a coordinated set of AI tools across search, wearables, clinical research, and genomic sequencing, to create a highly advanced multi-channel approach, as seen in the Exhibit 2, which aims to accelerate B2B business while addressing B2C by every means available.

EXHIBIT 2: Google is creating one of the most powerful multi-channel healthcare ecosystems in existence

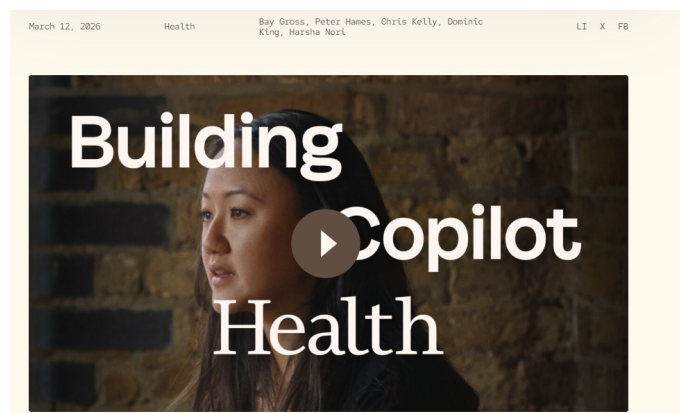


Source: Bernstein analysis

COPILOT HEALTH

Microsoft's Copilot Health is a dedicated, secure space inside Copilot that integrates and interprets an individual's health data. It is viewed as a "personal health cabinet" that aggregates all medical records taken from more than 50 wearables and app data, 50k hospitals in the US and lab tests. All the data are concentrated into one place with the aim of helping people to understand their health issue or "make sense of what they have" according to the promise.

EXHIBIT 3: Copilot Health



Source: Microsoft

The underlying idea is to increase patient awareness and make their time with the doctor more specific, insightful and useful. The intelligence that is put into Copilot enables the data to speak. Microsoft is rolling out Copilot Health and opened a waiting list mid-March due to the high level of interest it received on the Copilot alone. It varies from Dragon Copilot which addresses healthcare professionals by providing better workflow support.

WHY ISN'T AI SUITABLE YET FOR EVERYDAY USE IN MEDICAL PRACTICE? THE DIFFERENTIAL DIAGNOSIS

The differential diagnosis is used by medical practitioners to identify a specific disease and put a clear diagnosis on symptoms. This is an iterative process to which the inclusion and exclusion of criteria and factors enable the physician or medical staff to envisage a treatment or treatment pathway when a patient's symptoms can be attributed to multiple conditions. Symptoms of many diseases often overlap, and incorrect diagnoses can have negative repercussions for patient health.

Foundation of differential diagnosis

Various methods can be applied and taken into consideration: a) the symptoms complex (grouping of symptoms that commonly occur together), b) an anatomic approach to localize the possible organs involved, c) a systems approach (respiratory, digestive, cardiac, and the famous *VINDICATE* mnemonic for differential diagnosis: Vascular, Infectious, Neoplastic, Degenerative, Intoxication/Iatrogenic, Congenital, Autoimmune, Traumatic, Endocrine, Metabolic) to identify potential causes of symptoms; d) a mechanism approach, i.e. to understand the biology that could lead to a symptom; and e) an epidemiological approach by checking a patient's family history. The very first step is to obtain the anamnesis of the patient, or the medical evaluation of the patient's history and feedback.

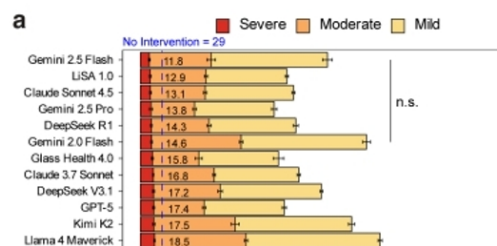
Governance prevents AI from assuming responsibility

AI is able to process the anamnesis, retrieve, share clinical data in a structured way, summarize guidelines and literature relevant to the case. It can even establish a probabilistic map and manage the meeting workflow in real time, but AI is not (yet) able to assess the nuances, contextual judgments or clinical risks of any medical decision. AI will never be able to assume medical responsibility in our view.

This is corroborated by the latest research from Stanford University, published in March ([NOHARM](#) - Numerous Options Harm Assessment for Risk in Medicine), which not just establishes a foundation for clinical safety evaluation at a moment when powerful LLMs are being integrated into patient care faster than the risks can be understood, it also outlines the harmful errors these models may do, compares the power of these models versus the average physician on a multidimensional performance evaluation (safety, completeness, i.e. adequate management plan) and examines the precision of the model (for example, exercising restraint or tending to avoid equivocal care). Amboss Lisa stood out for completeness and safety, largely outperforming the regular doctor's recommendation, while doctors remain best on precision Exhibit 5.

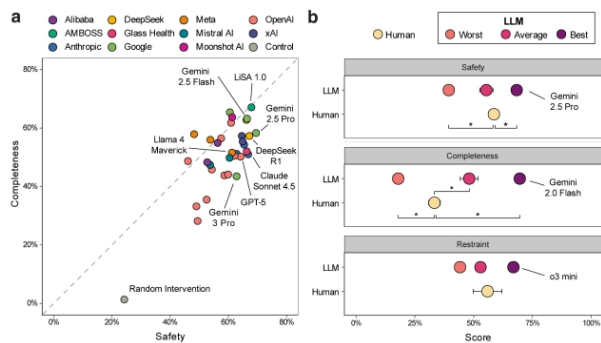
This research should lay the groundwork for the scalable and continuous oversight of patient safety as AI systems move from documentation support to clinical decision-making. However, while AI could be harnessed to achieve better patient outcomes, the journey remains complex.

EXHIBIT 4: Total harmful errors made by large models for the first 12 out of the 32 tested in the study, Gemini, Lisa and Claude are the best performer



Source: Stanford University, NOHARM study

EXHIBIT 5: Best models on safety and completeness are AMBOSS Lisa 1.0, followed by Gemini 2.5 pro and Glass Health 4.0, with scores ranging from 62.3% to 59%, while physicians are at 46% on average



Source: Stamford University, NOHARM study

BERNSTEIN TICKER TABLE

Ticker	Rating	Cur	14 May 2026	Price Target	TTM Rel. Perf.	Adjusted EPS				Adjusted P/E (x)		
			Closing Price			Cur	2025A	2026E	2027E	2025A	2026E	2027E
ERF.FP (Eurofins)	O	EUR	59.40	77.20	(4.0)%	EUR	3.88	4.06	4.68	15.3	14.6	12.7
AMBUB.DC (Ambu)	O	DKK	62.45	116.40	(52.8)%	DKK	2.28	2.39	2.94	27.4	26.1	21.2
LKFT.NA (Lakefront Biotherapeutics)	M	EUR	23.78	27.90	(12.3)%	EUR	1.12	4.87	(3.05)	21.2	4.9	(7.8)
GXI.GY (Gerresheimer)	U	EUR	25.14	18.90	(72.0)%	EUR	4.69	(0.57)	2.87	5.4	(44.0)	8.8
GETIB.SS (Getinge)	M	SEK	191.50	224.00	(12.9)%	SEK	11.29	12.30	13.59	17.0	15.6	14.1
SRT3.GY (Sartorius AG)	O	EUR	213.50	284.00	(20.9)%	EUR	4.79	5.80	7.07	44.6	36.8	30.2
DIM.FP (Sartorius Stedim)	O	EUR	163.10	245.00	(32.1)%	EUR	4.40	4.91	6.00	37.1	33.2	27.2
TECN.SW (Tecan)	M	CHF	133.20	160.80	(35.3)%	CHF	6.86	7.00	8.50	19.4	19.0	15.7
VIRP.FP (VIRBAC)	M	EUR	342.00	419.00	(0.4)%	EUR	17.93	19.34	20.93	19.1	17.7	16.3
EDME			1,526.12									

O - Outperform, M - Market-Perform, U - Underperform, NR - Not Rated, CS - Coverage Suspended

LKFT.NA, GXI.GY base year is 2024;

Source: Bloomberg, Bernstein estimates and analysis.

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